Neugen Laboratories
(A unit of Soulbeat Health & Infra LLP)
Regd. Office: Anant-2, Office No- 305, Nr. Kotecha Chowk,
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Neugen Laboratories
ELEVATING WELLNESS

TEST REPORT

Reg. No.	: 51008002786	**Reg. Date**	: 28-Oct-2025 12:02	**Collected On**	: 28-Oct-2025 12:02
Name	: Ms. KHUSHBU N RATHOD			**Reported Date**	: 28-Oct-2025 12:56
Age	: 13 Years	**Gender**	: Female **Ref. No. :**	**Dispatch At**	:
Ref. By	:			**Tele No.**	: 7874760209
Location	: JDF TRUST @ RAJKOT				

Test Name	Results	Units	Bio. Ref. Interval
HbA1c *HPLC on Bio-Rad D-10*	8.80	%	Normal: <= 5.6 Prediabetes:5.7-6.4 Diabetes: >= 6.5
Mean Blood Glucose *(Calculated)*	206	mg/dL	
EDTA Whole Blood			

Criteria for the diagnosis of diabetes

1. HbA1c >/= 6.5 * Or Fasting plasma glucose >126 gm/dL. Fasting is defined as no caloric intake at least for 8 hrs. Or
2. Two hour plasma glucose >/= 200mg/dL during an oral glucose tolerence test by using a glucose load containing equivalent of 75 gm anhydrous glucose dissolved in water. Or
3. In a patient with classic symptoms of hyperglycemia or hyperglycemic crisis, a random plasma glucose >/= 200 mg/dL. *In the absence of unequivocal hyperglycemia, criteria 1-3 should be confirmed by repeat testing.American diabetes association. Standards of medical care in diabetes 2011. Diabetes care 2011:34:S11.

Limitation of HbA1c

1) In patients with Hb variants even analytically correct results do not reflect the same level of glycemic control that would be expected in patients with normal population.
2) Any cause of shortened erythrocyte survival or decreased mean erythrocyte survival or decreased mean erythrocyte age eg. hemolytic diseases, pregnancy, significant recent/chronic blood loss etc. will reduce exposure of RBC to glucose with consequent decrease in HbA1c values.
3) Glycated HbF is not detected by this assay and hence specimens containing high HbF (>10%)may result in lower HbA1c values than expected. Importance of HbA1C (Glycated Hb.) in Diabetes Mellitus

- HbA1C, also known as glycated heamoglobin, is the most important test for the assessment of long term blood glucose control(also called glycemic control).
- HbA1C reflects mean glucose concentration over pas 6-8 weeks and provides a much better indication of longterm glycemic control than blood glucose determination.
- HbA1c is formed by non-enzymatic reaction between glucose and Hb. This reaction is irreversible and therefore remains unaffected by short term fluctuations in blood glucose levels.
- Long term complications of diabetes such as retinopathy (Eye-complications), nephropathy (kidney-complications) and neuropathy (nerve complications), are potentially serious and can lead to blindness, kidney failure, etc.
- Glyemic control monitored by HbA1c measurement using HPLC method (GOLD STANDARD) is considered most important. (Ref. National Glycohaemoglobin Standardization Program - NGSP)

Note : Biological reference intervals are according to American Diabetes Association (ADA) Guidelines.

Test done from collected sample.

This is an electronically authenticated report.

Approved by: **Dr. Mahesh Vidja**
M.D. (Path),
Reg No: G-19302

Generated On : 28-Oct-2025 14:24

Approved On: 28-Oct-2025 12:56

Patient report

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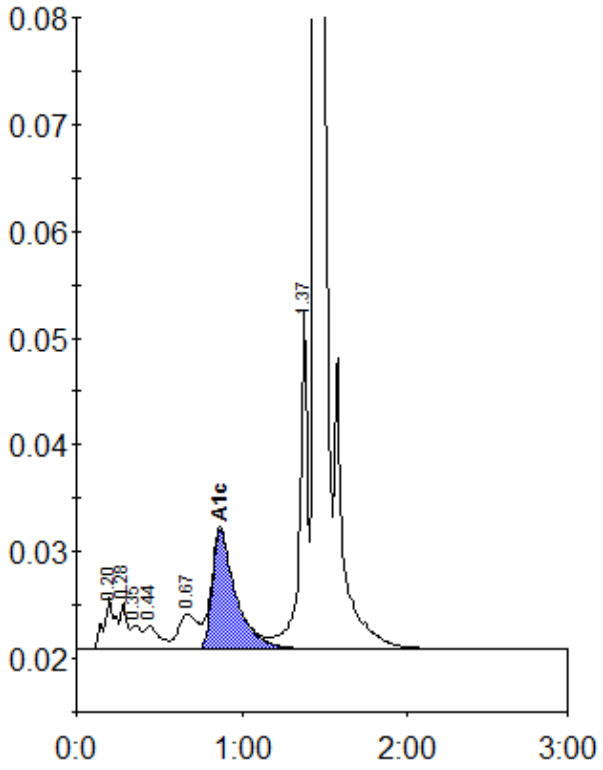
Bio-Rad	DATE: 28/10/2025
D-10	TIME: 08:20 AM
S/N: #D10B44857	Software version: 4.30-2
Sample ID:	151008002786
Injection date	28/10/2025 08:20 AM
Injection #: 27	Method: HbA1c
Rack #: ---	Rack position: 8



Peak table - ID: 151008002786

Peak	R.time	Height	Area	Area %
A1a	0.20	4813	23704	1.4
A1b	0.28	4463	12157	0.7
Unknown	0.35	2282	8941	0.5
F	0.44	2277	13929	0.8
LA1c/CHb-1	0.67	3321	26902	1.6
A1c	0.87	11239	111253	8.8
P3	1.37	32065	108784	6.6
A0	1.44	514036	1353375	81.6
Total Area:	1659045			

Concentration:	%
A1c	8.8

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Location	: JDF TRUST @ RAJKOT				

Test Name		Results	Units	Bio. Ref. Interval
C-PEPTIDE (FASTING)	L	**0.10**	ng/mL	0.4 - 2.1

C-Peptide (Fasting)

Serum
C Peptide is used for diagnostic workup of hypoglycemia:
-Diagnosis of factitious hypoglycemia due to surreptitious administration of insulin
-Evaluation of possible insulinoma
-Surrogate measure for the absence or presence of physiological suppressibility of endogenous insulin secretion during diagnostic insulin-induced hypoglycemia (C-peptide suppression test)
Assessing insulin secretory reserve in selected diabetic patients (as listed below) who either have insulin autoantibodies or who are receiving insulin therapy:
-Assessing residual endogenous insulin secretory reserve
-Monitoring pancreatic and islet cell transplant function
-Monitoring immunomodulatory therapy aimed at slowing progression of preclinical, or very early stage type 1 diabetes mellitus.

C-peptide (connecting peptide), a 31-amino-acid polypeptide, represents the midportion of the proinsulin molecule. Proinsulin resembles a hairpin structure, with an N-terminal and C-terminal, which correspond to the A and B chains of the mature insulin molecule, oriented parallel to each other and linked by disulfide bonds. The looped portion of the hairpin between the A and B chains is called C-peptide. During insulin secretion, C-peptide is enzymatically cleaved off and cosecreted in equimolar proportion with mature insulin molecules.

---------- End Of Report ----------

Test done from collected sample.

This is an electronically authenticated report.

Approved by: **Dr. Amit Rathod**
M.D. (Path) PDCC (Oncopath)

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